

MediKiosk — Loopholes & Solutions

AI-Powered Clinical History Software Platform — Risk Register & Mitigations

#	Loophole	Solution
1	Kiosk history-taking takes longer than the 2-5 min consult it replaces	Time-cap session (~3-4 min), auto-save partial history, decouple kiosk completion from queue position
2	Excludes elderly/low-literacy patients it claims to serve	Assisted/priority lane + staffed support (1 helper per N kiosks), not per-patient
3	ASR/OCR accuracy for handwriting & regional accents unproven	Confidence threshold + fallback to manual/staff entry when low-confidence
4	Regional/folk disease terms unrecognized by AI ontology (“peeliya”, “bawaseer”)	Region-specific synonym dictionary + always show raw patient statement to doctor, flagged if unmapped
5	AI errors (mis-transcribed allergy, missed red flag, alarm fatigue)	Doctor always reviews/edits before saving; tiered alert confidence; log all AI edits for audit
6	Consent is unrealistic for low-literacy patient in <2 min	Simplified audio consent + staff-assisted consent option, never solely self-serve for vulnerable users
7	Assumes hospital HIS/EMR compatibility	Universal fallback: exportable PDF/printout summary if HIS integration unavailable
8	AYUSH history (Dashavidha Pariksha) bolted on, doubles complexity	Separate AYUSH module/timeline, not merged into same engine as allopathic flow
9	No plan for hardware cost, power/network outage, shared-device hygiene	Offline-first local caching + resume-on-reconnect; sanitizable hardware (touchless/voice-first)
10	Doctors may not trust/reuse AI summary, re-ask everything anyway	Pilot with opt-in doctors first, show time-saved data, summary editable not prescriptive
11	Claims/stats asserted without sourcing	Cite every claim; benchmark kiosk's own time against consult-time savings claimed
12	One family/multiple patients per session	Support multi-profile sessions per queue token
13	Patient can't operate touchscreen at all (zero digital literacy)	Voice-first mode as default fallback, not just supplementary
14	Patient's spoken history contradicts old digitized records	AI flags contradictions for doctor (“patient says no diabetes, prior report says yes”)
15	Contagious patients sharing kiosk hardware	Voice-first/touchless design; wipeable surfaces; sanitizer stations at kiosk
16	Patients with disabilities (speech/motor/vision impaired)	Accessibility mode: pure-voice, pure-touch, or staff-assisted fallback paths
17	Not enough kiosks → queue-before-the-queue	Cap kiosk use as optional/parallel to doctor queue, never gating; scale kiosk count to load-test data
18	Emotionally distressed patients can't do structured Q&A	“Skip to human” escape button at every step
19	Conflicting data across old documents	Show conflicts side-by-side to doctor, don't auto-resolve

#	Loophole	Solution
20	Patient corrects/changes answer mid-session	Voice-editable “go back” command in dialogue flow
21	Network/server drops mid-session	Local device caching, auto-resume from last answered question
22	Minors/unconscious patients can't self-consent	Guardian/proxy ABHA authentication and consent flow